Name	:	**Manjunatha G M**	**PET No**	:	**KMIO202411951**
Age/Sex	:	**44 Years/ Male**	**UHID**	:	**20240007016**
Ref/Dept	:	**Surgical Oncology**	**Date**	:	**13.12.2024**

^{18}F FDG PET-CT Report

CLINICAL INDICATION: Case of periampullary carcinoma status post stenting.

TECHNIQUE: Whole body images (vertex to mid thigh) were acquired in 3-D mode 60 min after i.v. injection of 4.39mCi of 18F-FDG using **a uMI 550 80 slice Digital PET CT. SIPM with TOF detector to crystal coupling.** FBS before trace injection was 130mg/dl. IV contrast was used. Reconstruction of the acquired data was performed so as to obtain fused PET-CT images in all views.

FINDINGS:

Abdomen and Pelvis:

- Post surgical changes noted in periampullary region. Biliary stent noted in situ. No evidence of FDG avid lesion at surgical site.
- **FDG uptake is seen in portocaval lymph nodes (largest measuring 1.8 x 1.3 cm SUVmax 4.5).**
- Liver appears enlarged (size 19.9 cm). No evidence of FDG avid lesion in liver. Spleen appears normal. Gall bladder appears normal with no evidence of calculus.
- **FDG uptake is seen in paraaortic lymph nodes in retroperitoneum (size 1.1x 1.1 cm SUVmax 6.5).**
- Stomach and small intestine appear normal. Both kidneys and adrenals appear normal.
- **FDG uptake is seen in right external iliac lymph node (size 2.2 x 1.5 cm SUVmax 15.5).**

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- Prostate gland appears unremarkable. Urinary bladder appears normal. No evidence of free fluid.

Head and neck:

- No evidence of FDG avid cervical lymph nodes.
- Pharyngeal mucosal space, larynx, thyroid and the cervical trachea appear normal. Parotid glands and submandibular glands are unremarkable.
- No focal mass lesion/mass effect seen within the visualized brain parenchyma.

Chest:

- Both lungs appear normal with no evidence of nodules. No evidence of pleural effusion/ thickening.
- No evidence of FDG avid mediastinal and axillary lymphadenopathy.
- Esophagus appears normal. Physiological tracer uptake is noted in myocardium.

Musculoskeletal system:

- No abnormal FDG uptake is seen in bones of axial and appendicular skeleton. No evidence of lytic or sclerotic lesions.

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IMPRESSION:

- ***Post surgical changes in periampullary region. Biliary stent in situ. No evidence of FDG avid mass lesion at surgical site.***
- ***Hypermetabolic portocaval, retroperitoneal and right external iliac lymph node, of concern for suggestive of metastasis. Suggested biopsy correlation.***
- ***No other hypermetabolic lesions in the body in this study.***

Dr. Harish. B DRM, DNB , FEBNM, PGDHA/HM
Professor and Head,
Department of Nuclear Medicine

Dr. Akash Ringane
Consultant
Department of Nuclear Medicine

Please note: Routine whole body PET-CT Oncology protocol does not include lower limbs. It is included only on Clinician request or as per clinical status. PET CT is not sensitive for brain metastasis and MRI is indicated whenever there is suspicion / clinically warranted.

S/o disease progression

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